

SIR CHARLES GAIRDNER OSBORNE PARK
HEALTH CARE GROUP

BENZODIAZEPINE WITHDRAWAL MANAGEMENT

QUICK REFERENCE GUIDE

URN:

Family name:

Given name(s):

Gender:

DOB:

Signs of intoxication	Signs of withdrawal		
	Onset	Common	
Poor motor coordination Slurred speech Disinhibition Low blood pressure Poor concentration Mood instability Sedation	1–10 days after last dose Lasts 3–6 weeks (depending on the half-life of the benzodiazepine)	Mental State: Anxiety / panic Insomnia / vivid dreams Restlessness / agitation Irritability Poor concentration / memory Depressed mood Feelings of depersonalisation	Physical Symptoms: Dry retching / Nausea Decreased appetite Aches and pains Headaches Palpitations Tremor Blurred vision Increased temperature Ataxia Menstrual changes
		Uncommon (severe)	
		Delirium, Delusions, Paranoia, Hallucinations, Catatonia, Seizures	
Assessment / management tools (once diagnosis established)			
N/A			
Acute withdrawal treatment models			
General principles		Inpatient management	
Tolerance to benzodiazepines develops quickly Most benzodiazepine withdrawal is managed in the community with GP or specialist AOD service involvement Refer patients on an opioid treatment program back to their prescribers Do not prescribe to patients with whom you are not familiar Check the Medicare prescription shopping hotline Phone 1800 631 181 OR WA Health S8 prescriber information service Phone 08 9222 4424		Acute withdrawal is managed by dose stabilisation whilst in hospital and then referred for management in the community. Clinical judgement on dose is required as history may be unreliable. Specialist advice is recommended for polysubstance users If there is no hepatic impairment, convert daily dose to a diazepam-equivalent dose. Start with a dose reduction of 20–40% divided QID (or 80mg/day whichever is lower. If uncertain, a conservative starting dose of 10mg QID is usually adequate, with dose adjustment based on symptoms Dose reduction should be slow aiming for 10% reduction every 1–2 weeks. Reduction rate can be slowed if preferred by the patient (Grade B)	
Post-withdrawal treatment options			
Ongoing counselling and support groups		Involve GP/AMS and significant others	

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If the patient is not improving on this regimen, seek assistance from specialist AOC Clinicians or the Drug and Alcohol Advisory Service (DACAS).

Benzodiazepine Equivalents					
Name	*Duration of Action	Approximate dose equivalent to 5mg Diazepam	Trade Names	Tablet Strengths	Schedule
Alprazolam	Short	0.5 – 1mg	Alprax, Kalma, Xanax, Ralozam	0.25mg, 0.5mg 1mg, 2mg	Schedule 8 Controlled Drug
Bromazepam	Intermediate	3 – 6mg	Lexotan	3mg, 6mg	S4R
Clobazam	Long	10 – 15mg	Frisium	10mg	S4R
Diazepam	Long	5mg	Antenex, Ducene, Ranzepam, Valium, Valpam	2mg, 5mg	S4R
Flunitrazepam	Long	1 – 2mg	Hypnodorm	1mg	Schedule 8 Controlled Drug
Lorazepam	Intermediate	0.5 – 1mg	Ativan	1mg, 2.5mg	S4R
Nitrazepam	Long	5mg	Alodorm, Mogadon	5mg	S4R
Oxazepam	Short	15 – 30mg	Alepam, Murelax, Serepax	15mg, 30mg	S4R
Temazepam	Short	10 – 20mg	Normison, Temaze, Temtabs	10mg	S4R
Triazolam	Very short	0.25mg	Halcion	0.125mg	S4R
Zolpidem	Very short	10mg	Dormizol, Somidem, Stildem, Stilnox, Zolpibell	10mg (6.25mg & 12.5mg modified release)	S4R
Zopiclone	Very short	7.5mg	Imovane, Imrest	7.5mg	S4R

*Approximate duration of action	Approximate Time
Very Short	Less than 6 hours
Short	6 – 12 hours
Intermediate	12 – 24 hours
Long	Greater than 24 hours

Reference: Project, W. W. M. (2021). Alcohol and Other Drugs Withdrawal Management: Practice and Pathways, Government of Western Australia. Department of Health: 17–18.